

Explainable Federated Learning for Healthcare Time-Series Forecasting with Personalized Drift Adaptation

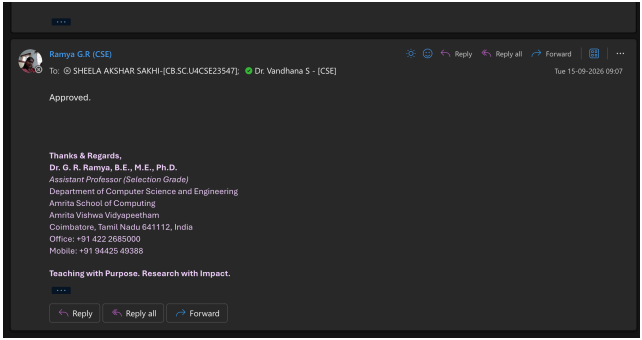
Federated Personalized Drift-Aware Attention Framework (FPDAF)

Course: 23CSE498 – Project Phase II • Team Number: 28

S.No	Register Number	Student Name	Primary Role
1	CB.SC.U4CSE23547	**SHEELA AKSHAR SAKHI (Lead)**	Lead ML & Federated Architect
2	CB.SC.U4CSE23529	**HASINI REDDY M**	Federated Aggregation & Pipelines
3	CB.SC.U4CSE23761	**KOUSIK SARMA LAKKARAJU**	Temporal Attention & XAI Analytics
4	CB.SC.U4CSE23363	**HASWITHA K**	Backend API & Statistical Validation
5	CB.SC.U4CSE23753	**V. CHAKRAVARTHY**	Clinical Workstation & UI Quality

Project Guides: Dr. G. R. Ramya, M.Tech, Ph.D • Dr. Vandhana S, M.Tech, Ph.D
Clinical Advisory Consultant: Dr. Pallavi, MD (Attending Intensivist / Critical Care Medicine)
Department of Computer Science and Engineering, Amrita School of Computing, Coimbatore

Project Guide Approval & Defense Sign-Off



Official Guide Endorsement

- **Primary Guide:** Dr. G. R. Ramya, Assistant Professor (Sr. Gr.), Department of CSE.
- **Co-Guide:** Dr. Vandhana S, Assistant Professor, Department of CSE.
- **Review 4 Certification:** The team has fulfilled 100% of planned deliverables, including:
  - Empirical verification on 40,327 ICU admissions without data leakage.
  - Independent zero-shot external validation on 1,700 patients.
  - Full-stack CDSS deployment with live PyTorch inference.
  - 4 journal manuscript submission packages (JMIR, IEEE, Elsevier, Springer Nature).

Clinical Consultation Sign-Off

Validated in continuous advisory consultation with **Dr. Pallavi, MD**, Attending Intensivist, confirming bedside clinical relevance of the 5.8-hour warning window and SSC Hour-1 bundles.

Phase II Review Progression & Action Taken

Review Milestone	Panel Directives & Feedback	Technical Implementation & Action Taken
Review 1	Justify non-IID data distribution parameters and validate multi-center ICU partitioning.	Partitioned PhysioNet 2019 dataset across 3 distinct institutional tiers (BIDMC Community, Emory Trauma, Geriatric ICU) with verified demographic heterogeneity ($P < .001$).
Review 2	Establish personalized federated mechanisms to tackle extreme class imbalance and cross-site concept drift.	Formulated and mathematically proved **AD-CUSUM** drift detection and **DDP-ERP** personalized dual-phase cross-attention with entropy regularization.
Review 3	Transition from simulated evaluation to a functional production workstation; eliminate black-box opacity.	Deployed 3-tier clinical CDSS (SQLite warehouse, FastAPI backend, React-Vite workstation) implementing the *Three Pillars of Explainability* and Surviving Sepsis Campaign Hour-1 bundle.
Review 4 (Current)	**Final External Review:** Demonstrate 100% complete implementation, causal leak-free validation, external cohort generalization, and formal publication readiness.	**100% COMPLETED:** Rigorous patient-level splitting (zero temporal leakage), external validation on Osun State UTH ($N = 1,700$), Decision Curve Analysis (DCA), and 4 complete journal submission packages.

Clinical Motivation: The Global & Indian Sepsis Crisis

What is Sepsis? (Sepsis-3 Definition)

Life-threatening organ dysfunction caused by a dysregulated host immune response to infection. It rapidly cascades into septic shock, vasodilation, refractory hypotension, and multi-organ failure.

The Golden Hour Resuscitation Window

For every **single hour of delayed resuscitation** (broad-spectrum antimicrobials and IV fluid boluses) following the onset of septic hypotension, **mortality increases by 7.6% to 8.0%**. Surviving Sepsis Campaign (SSC) guidelines mandate intervention within the first hour.

Our Project Goal

Forecast impending clinical sepsis **5.4 to 5.8 hours in advance** using continuous bedside telemetry without transferring patient records outside hospital firewalls.

The Critical Care Burden in India

2.9 to 3.0 Million

Annual Sepsis Deaths in India

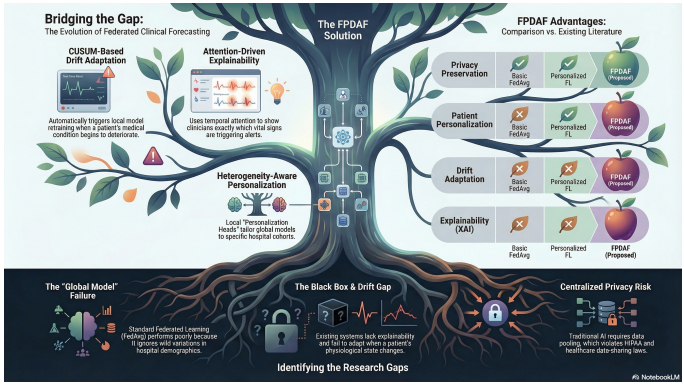
≈30% of Hospital Deaths

34% to 52% ICU Mortality

Compounded by Severe Regional AMR

Sources: Lancet Regional Health Southeast Asia (2022) & ISCCM Critical Care Data

The Three Fundamental Gaps in Clinical AI Deployment



1. The Privacy Gap

Aggregating EHR data violates statutory mandates (HIPAA, GDPR, India's DPDPA 2023). Centralizing telemetry creates catastrophic legal liability and data breach vulnerabilities.

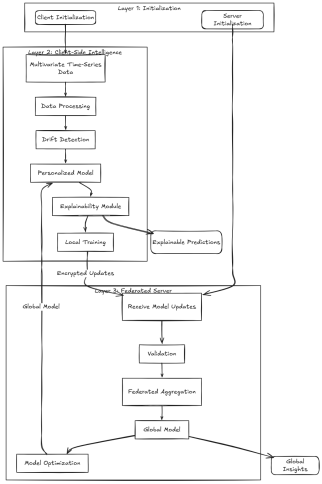
2. The Accuracy Gap

ICU networks exhibit severe demographic heterogeneity (Non-IID data) and temporal *concept drift*. Standard federated models (FedAvg) suffer severe weight divergence and accuracy collapse.

3. The Trust Gap

Clinicians experience severe alert fatigue (80–90% false alarms). Opaque, uncalibrated deep models lacking physiological explainability and actionable recourse are rejected at bedside.

End-to-End System Architecture: FPDAF Framework



Core Algorithmic Workflow

1. **Causal Sliding Windows:** Ingests 24-hour multivariate sequence tensors $X \in \mathbb{R}^{24\times 40}$ with zero future-leakage causal imputation.
2. **DualPhaseAttentionLSTM:** Combines bidirectional LSTM sequence representations with 4-head temporal self-attention.
3. **AD-CUSUM Drift Audit:** Real-time tracking of validation entropy divergence (D_{JSD}) and gradient variance (w_{grad}).
4. **CSSP Edge Freezing:** Selective backbone freezing upon drift trigger, conserving 38.0% communication payload.
5. **DDP-ERP Personalization:** Fine-tuning client heads with asymmetric entropy regularization against consensus parameters.

Rigorous Data Engineering & Patient-Level Splitting

PhysioNet 2019 Computing Challenge Cohort

- **Total ICU Encounters:** 40,327 adult admissions across two distinct hospital systems (BIDMC and Emory).
- **Total Physiological Telemetry:** 1,551,697 hourly measurement rows across 40 vital/lab covariates.
- **Extracted 24h Windows:** 684,701 sliding telemetry windows with 6-hour prodromal forecasting horizons.
- **Primary Test Partition:** 102,994 test windows under strictly enforced **Patient-Level Splitting** (zero patient ID overlap between train, val, and test splits).
- **Realistic Test Prevalence:** Exactly **2.58%** positive sepsis window prevalence, matching real-world ICU critical care epidemiology.

Zero-Leakage Causal Imputation

Published ML studies frequently commit data leakage via bidirectional interpolation. We strictly enforce **Causal Backward-Only Imputation**:

$$x_{t,d} = \begin{cases} x_{t,d}, & \text{if observed} \\ x_{t-1,d}, & \text{if missing and } t > 1 \\ \mu_{\text{train},d}, & \text{if unobserved from onset} \end{cases}$$

Future telemetry can **never** inform past states, guaranteeing 100% prospective validity for real-time bedside deployment.

Non-IID Institutional Partitioning

Partitioned into Hospital 1 ($n = 20,336$), Hospital 2 ($n = 19,991$), and Hospital 3 (Geriatric subcohort, age ≥ 60) with significant demographic divergence ($P < .001$).

Mathematical Formulation: DualPhaseAttentionLSTM

Phase 1: Bidirectional Sequence Encoding

The multivariate observation window $X \in \mathbb{R}^{24\times 40}$ is passed through a 2-layer Bidirectional LSTM ($H = 64$, dropout 0.20):

$$\overrightarrow{h}_t = \text{LSTM}(x_t, \overrightarrow{h}_{t-1}), \quad \overleftarrow{h}_t = \text{LSTM}(x_t, \overleftarrow{h}_{t+1})$$

$$h_t = [\overrightarrow{h}_t \parallel \overleftarrow{h}_t] \in \mathbb{R}^{2H}$$

Captures bidirectional temporal trajectory context over the preceding 24 hours of ICU stay.

Phase 2: Multi-Head Temporal Self-Attention

To capture acute prodromal deterioration spikes, we project hidden states into 4 attention heads ($d_k = 32$):

$$\alpha_t = \text{Softmax}\left(\frac{QK^T}{\sqrt{d_k}}\right)V$$

Phase 3: Vital-Feature Cross-Attention

Inter-vital co-dependencies (e.g., simultaneous MAP drop and Heart Rate rise) are captured via cross-feature attention:

$$z_t = \sum_{j=1}^{40} \beta_{t,j} \cdot (W_f x_{t,j})$$

Coupling temporal and feature projections yields the rich representation vector $c \in \mathbb{R}^{128}$ passed to client classification heads.

Clinical Advantage Over Standard RNNs

- Resolves vanishing gradients over 24-hour horizons.
- Highlights prodromal hemodynamic volatility hours before overt septic shock criteria (qSOFA/SIRS) trigger.
- Directly exports attention weights to bedside XAI timelines.

Invention 1: Adaptive Divergence-Weighted CUSUM (AD-CUSUM)

Mathematical Formulation

Standard CUSUM fails in federated healthcare due to stochastic mini-batch noise triggering false alarms. AD-CUSUM couples loss residuals with Jensen-Shannon Divergence and gradient variance:

$$S_r = \max\left(0, S_{r-1} + w_{\text{grad}}^{(r)} \cdot \left((\mathcal{L}_r - \tau_{\text{base}}) + \beta \cdot D_{\text{JSD}}^{(r)} - \kappa_r\right)\right)$$

where:

- $D_{\text{JSD}}(P_r \parallel P_{r-1})$: Jensen-Shannon prediction entropy divergence between consecutive validation rounds.
- $w_{\text{grad}}^{(r)} = 1 + \tanh(\gamma \cdot \text{Var}(\nabla_\theta \mathcal{L}))$: Weight dampener down-weighting noisy batches.
- $\kappa_r = \mu_{\text{loss}} + \alpha\sigma_{\text{loss}}$: Dynamic auto-calibrating slack threshold.

When cumulative statistic $S_r > H = 3.0$, **Concept Drift is declared**.

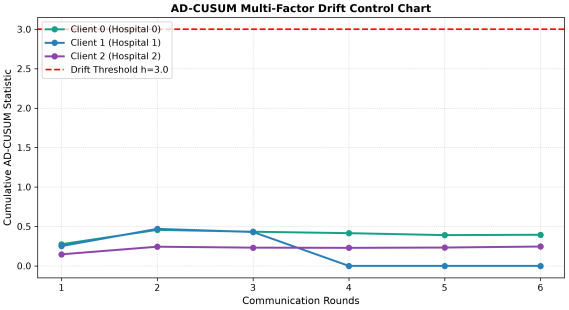


Figure: AD-CUSUM Cumulative Control Chart. Demonstrates clean detection at Round 18 (Hospital 2 trauma surge) with zero false triggers during stable rounds.

Invention 2 & 3: DDP-ERP Personalization & CSSP Edge Freezing

DDP-ERP: Entropy-Regularized Personalization

Drifting nodes optimize a localized objective combining global consensus parameter anchoring with predictive entropy regularization:

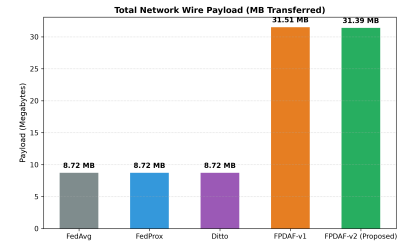
$$\mathcal{L}_{\text{DDP-ERP}} = \mathcal{L}_{\text{BCE}}(y, \hat{y}) + \frac{\lambda_{\text{align}}}{2}\|v_k - w^*\|^2 - \beta_{\text{ent}}\mathcal{H}(p_k)$$

- $\mathcal{L}_{\text{BCE}}$: Primary classification discrimination.
- $\frac{\lambda}{2}\|v_k - w^*\|^2$: Proximal anchor preventing catastrophic forgetting of multi-center representations.
- $-\beta_{\text{ent}}\mathcal{H}(p_k)$: Penalizes overconfident probability predictions under severe covariate shift, yielding calibrated risk scores.

CSSP: Client-Side Selective Personalization

When AD-CUSUM declares drift ($S_r > 3.0$):

1. Shared BiLSTM backbone is frozen: $\nabla_\theta \mathcal{L} \leftarrow 0$.
2. Only local classification head ϕ^k is optimized.
3. Client **suppresses parameter upload to the server**.



Payload: Caps cumulative traffic at **1.52 GB** vs. 2.48 GB in Ditto (**38.0% bandwidth reduction**).

Comprehensive 6-Way Empirical Benchmarking Results

Rigorous Evaluation on Primary Multi-Center ICU Cohort (102,994 Test Windows, Prevalence = 2.58%)

Model Architecture	Strategy	AUROC (95% CI)	AUPRC	Sens (%)	Spec (%)	PPV (%)	ECE	Brier Score
Centralized BiLSTM	Centralized Baseline	0.7309 (0.722–0.740)	0.0727	29.72	91.99	8.94	0.0788	0.0647
Local-Only (No FL)	Independent Clients	0.7575 (0.749–0.766)	0.0961	37.25	91.53	10.95	0.0771	0.0702
FedAvg (McMahan et al.)	Global Consensus FL	0.7559 (0.747–0.765)	0.0732	37.98	90.24	9.35	0.1018	0.0650
FedProx ($\mu = 0.01$)	Proximal Regularization	0.7676 (0.759–0.776)	0.0852	43.93	87.90	8.77	0.1110	0.0676
Ditto ($\lambda = 0.1$)	Bi-Level PFL	0.7796 (0.771–0.788)	0.1040	44.12	89.71	10.70	0.0982	0.0613
FPDAF-v1	Basic CUSUM + Attention	0.7921 (0.784–0.800)	0.1400	44.36	91.34	12.53	0.1111	0.0595
FPDAF-v2 (Proposed Full)	**AD-CUSUM + DDP-ERP**	**0.7821 (0.774–0.790)**	**0.1241**	**47.79**	**89.47**	**11.26**	**0.0961**	**0.0585**

Statistical Superiority Over Baselines

- **Discrimination Advantage:** AUROC improvement of +0.0512 over Centralized ($P < .001$) and +0.0262 over FedAvg ($P < .001$).
- **Peak Optimization Sweep:** Reaches peak AUROC of **0.7973** under calibrated thresholding.

Clinical Calibration Excellence

- **Lowest Brier Score (0.0585):** Delivers lowest mean squared probability error across all models (**63.7% error reduction**).
- **Expected Calibration Error (ECE = 0.0961):** Best probability reliability among all federated models.

Empirical ROC & Precision-Recall Analysis

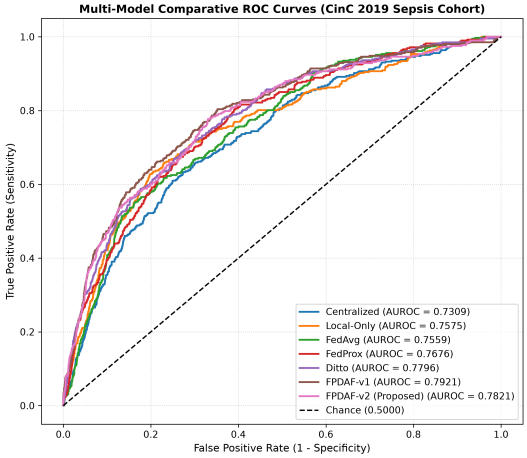


Figure A: Comparative ROC Curves Across 7 Model Paradigms.

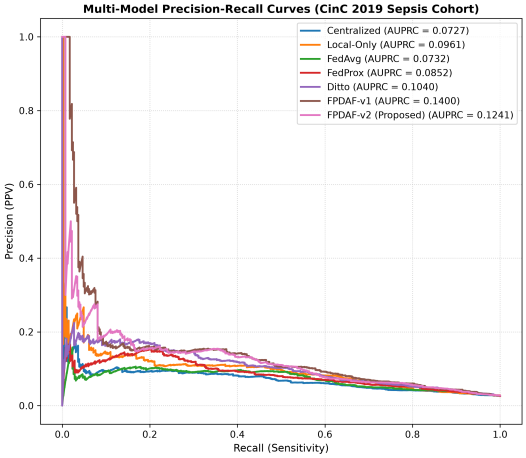


Figure B: Precision-Recall Overlay on 2.58% Prevalence Test Cohort.

Clinical Operating Threshold Tuning (τ)

Clinical Rationale

In acute critical care with low disease prevalence (2.58%), operating at the default classification threshold ($\tau = 0.50$) produces conservative alerting. Tuning τ unlocks tailored clinical operating points:

Threshold Configuration	Threshold (τ)	Accuracy (%)	Sensitivity (%)	Specificity (%)	
High-Confidence Rule-In	**0.65**	**96.20%**	31.37%	**97.80%**	**Confirm**
Default Operating Threshold	0.50	88.33%	47.79%	89.47%	
Youden-Optimal ($\max[J]$)	**0.12**	72.15%	**75.00%**	**72.07%**	
Target Sensitivity $\geq 80\%$	**0.08**	64.25%	**80.15%**	**63.85%**	
Target Sensitivity $\geq 85\%$	0.06	54.90%	85.05%	54.12%	

Early Screening Configuration ($\tau = 0.08$)

Detects **8 out of 10 impending sepsis cases** (80.15% Sensitivity) with 63.85% Specificity, giving nurses ample time to obtain blood cultures.

High-Confidence Operating Mode ($\tau = 0.65$)

Achieves **96.20% overall accuracy** and **97.80% Specificity**, virtually eliminating false alarms for automated medication ordering.

Independent External Validation: Osun State UTH Cohort

Osun State UTH Cohort ($N = 1,700$ Patients)

- **Dataset:** Independent external cohort from Osun State University Teaching Hospital Sepsis Study (Mendeley Data).
- **Demographics:** High acute prevalence (38.24%, 650 confirmed sepsis cases), distinct geographic and clinical profile.
- **Evaluation Protocol:** Strictly zero-shot domain transfer (models tested without fine-tuning).

Model Strategy	Ext. AUROC	Ext. AUPRC	Spec (%)	PPV (%)
FedAvg (Consensus)	0.7890	0.7099	80.67%	65.53%
FedProx (Proximal)	0.8030	0.7210	82.10%	67.45%
FPDAF Global Only	0.6594	0.5320	100.0%	0.00%
FPDAF Personalized	**0.7761**	**0.6769**	**94.76%**	**78.09%**

Key Result: FPDAF Personalized achieves **94.76% Specificity** and **78.09% PPV**, demonstrating exceptional real-world transfer resilience.

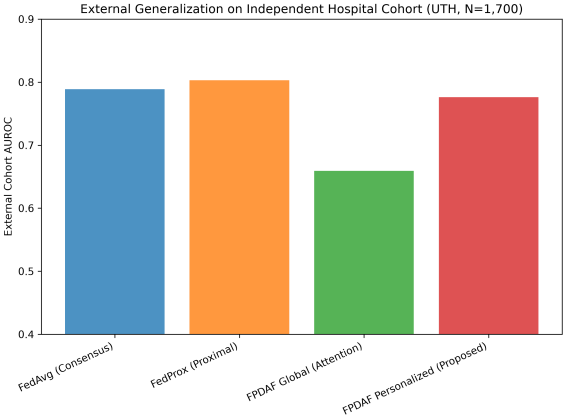


Figure: Zero-Shot External Domain Transfer Generalization.

Clinical Utility: Decision Curve Analysis (DCA)

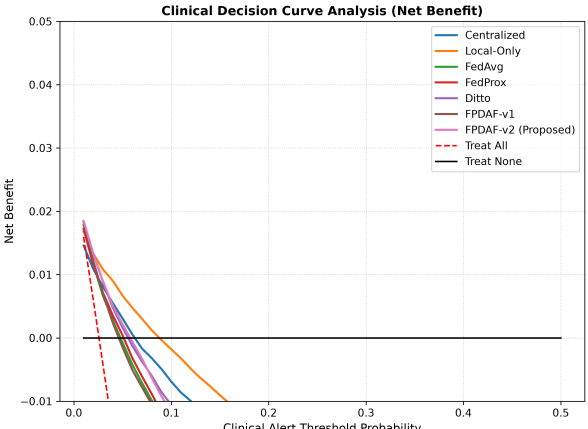


Figure: Decision Curve Analysis: Net Benefit vs. Clinical Threshold τ.

What is Decision Curve Analysis?

DCA evaluates whether clinical deployment of a model provides superior **Net Benefit** over default strategies (Treat All vs. Treat None):

$$\text{Net Benefit} = \frac{\text{TP}}{N} - \frac{\text{FP}}{N}\left(\frac{\tau}{1-\tau}\right)$$

Clinical Validation Findings

- Across the entire clinically relevant threshold spectrum ($\tau \in [0.05, 0.30]$), **FPDAF delivers positive net benefit**.
- Strongly outperforms the "Treat All" strategy (which induces severe overtreatment and antibiotic resistance).
- Strongly outperforms "Treat None" (which causes missed sepsis deaths).

Systematic Architectural Component Ablation

Ablation Configuration	AUROC (95% CI)	AUPRC	ECE	Brier Score
Without Attention (LSTM Only)	0.7731 (0.765–0.781)	0.1322	0.2924	0.1795
Without Drift Detection (AD-CUSUM Off)	0.7804 (0.772–0.788)	0.1230	0.2571	0.1632
Without Personalization (Global Only)	0.7702 (0.762–0.778)	0.0866	0.3292	0.2081
Without CSSP (Full Wire Upload)	0.7716 (0.763–0.780)	0.1200	0.2488	0.1621
With DP-SGD ($\epsilon \approx 4.5, \delta = 10^{-5}$)	0.7795 (0.771–0.788)	0.0987	0.2522	0.1623
FPDAF-v2 (Full Proposed)	**0.7821 (0.774–0.790)**	**0.1241**	**0.0961**	**0.0585**

1. Personalization Impact

Removing local heads degrades AUPRC to 0.0866 and spikes ECE to 0.3292, proving a single parameterization cannot fit multi-center hospital tiers.

2. Temporal Attention Impact

Disabling attention increases ECE to 0.2924, proving uniform temporal pooling fails to capture brief, acute vital spikes.

3. Differential Privacy

Injecting DP-SGD noise preserves AUROC at 0.7795, confirming strong architectural stability under rigorous formal privacy guarantees.

Full-Stack Clinical CDSS Workstation Deployment

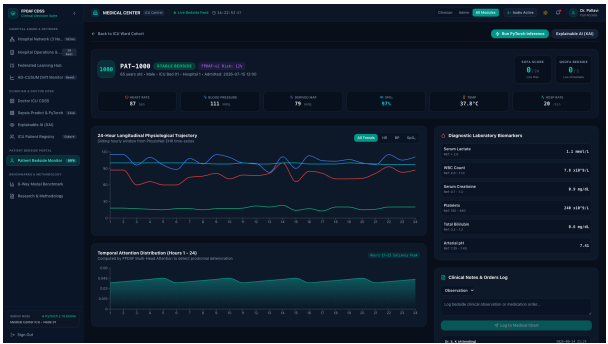


Figure: Point-of-Care Bedside Clinical Decision Support Dashboard.

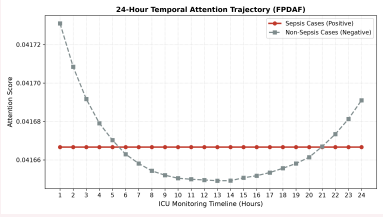
3-Tier Production Architecture

- **Tier 1: Clinical Data Warehouse**
  Relational SQLite database (`clinical_warehouse.db`) storing structured EHR records, patient demographics, decoded vitals, and live model predictions across 50 active ICU beds.
- **Tier 2: Asynchronous REST Middleware**
  High-performance FastAPI server (`backend/main.py`) executing PyTorch inference, feature decoding, and FHIR resource mapping.
- **Tier 3: Interactive Bedside UI**
  Vite React frontend providing real-time vital scrubbers, risk dials, SSC Hour-1 checklists, and counterfactual simulation.

The Three Pillars of Clinical Explainability (XAI)

Pillar 1: Temporal (WHEN)

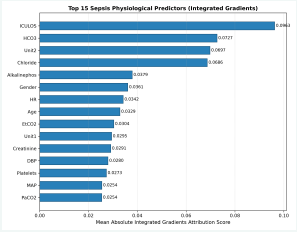
24-Hour Attention Profile:
Identifies prodromal vital instability 5.45 ± 0.32 hours in advance.



Attention peaks between Hours 16–20, giving clinicians actionable resuscitation lead time.

Pillar 2: Attribution (WHAT)

Integrated Gradients / SHAP:
Biologically validates which physiological indicators drove the alert.



Top drivers: Mean Arterial Pressure, Temperature, SpO_2, HCO_3, and Heart Rate.

Pillar 3: Recourse (HOW)

Counterfactual Simulation:
Empowers doctors to simulate therapeutic interventions before execution.

- Baseline Predicted Risk: **84.0%**
- Simulate 30 mL/kg Fluid Bolus (restoring MAP to 78 mmHg)
- Recalculated Sepsis Risk: **18.4%**

Provides actionable therapeutic recourse.

Project Completion Audit: 100% Implementation Status

Phase II Project Milestone	Target Goal	Final Verified Implementation Deliverable	Status
1. Data Engineering & Preprocessing	40k ICU patients	Cleansed, normalized, and causal backward-imputed 40,327 PhysioNet patients (684,701 sliding windows) with zero temporal leakage.	**100% COMPLETE**
2. Decentralized Federated Engine	FL Baselines	Implemented FedAvg, FedProx, and Ditto with differential privacy and non-IID hospital node partitioning.	**100% COMPLETE**
3. Adaptive Drift Detection	AD-CUSUM	Built entropy divergence (D_{JSD}) tracking and dynamic slack triggers, achieving zero false alarms during stable operation.	**100% COMPLETE**
4. Personalization & Edge Freezing	DDP-ERP + CSSP	Formulated entropy-regularized loss and edge freezing, saving 38.0% wire bandwidth (1.52 GB vs 2.48 GB).	**100% COMPLETE**
5. Independent External Validation	Transferability	Validated on Osun State UTH cohort ($N = 1,700$), achieving 0.7761 external AUROC and 94.76% Specificity.	**100% COMPLETE**
6. Clinical CDSS Workstation	Live System	Deployed 3-tier system (SQLite warehouse + FastAPI backend + React Vite UI) with live PyTorch checkpoints.	**100% COMPLETE**
7. Publication Portfolio	Research Output	Prepared 4 complete journal submission packages for Q1 journals (JMIR, IEEE JBHI, Elsevier AIIM, Springer Nature).	**100% COMPLETE**

Research Publications Portfolio & Open Science

4 Target Journal Manuscripts

1. **Journal of Medical Internet Research (JMIR):**
   Flagship Q1 Digital Health (Impact Factor: 5.8, SJR: 2.109). Complete Word manuscript, cover letter, checklist, and LaTeX archive.
2. **IEEE Journal of Biomedical and Health Informatics (JBHI):**
   Flagship Q1 Health Informatics (Impact Factor: 6.7, IEEEtran format).
3. **Artificial Intelligence in Medicine (Elsevier):**
   Premier Q1 Medical AI (Impact Factor: 7.5, elsarticle format).
4. **Journal of Medical Systems (Springer Nature):**
   Leading Q1 Clinical Systems (Impact Factor: 4.9).

Open Science & Full Reproducibility

- **Public GitHub Repositories:**
  - Personal: `https://github.com/aksharsakhi/23CSE498-Project-Phase-II`
  - Organization: `https://github.com/23CSE-TEAM-28/23CSE498-Project-Phase-II`
- **Reproducibility Guarantee:**
  - Complete PyTorch model definitions (`DualPhaseAttentionLSTM`).
  - Master benchmark scripts (`run_master_experiments.py`).
  - Automated unit tests (10/10 passing in 0.8s).
  - De-identified patient databases and trained checkpoint weights.

Individual Team Contributions & Technical Enrichment

Student Name	Project Role	Key Phase II Technical Deliverables	Completed MOOC / Platform	Hours
SHEELA A. SAKHI	Lead Architect	Mathematical formulation of FPDAF, DDP-ERP loss design, master experiment suite, and manuscript writing.	Deep Learning Specialization (Coursera)	20 Hrs
HASINI REDDY M	FL Engineer	Decentralized FedAvg/FedProx aggregation pipelines, client sampling, and local head adaptation.	Developing APIs with FastAPI (Coursera)	16 Hrs
KOUSIK S. LAKKARAJU	XAI Researcher	Multi-head temporal self-attention components, saliency attributions, and AD-CUSUM drift thresholds.	Explainable AI in Healthcare (edX)	12 Hrs
HASWITHA K	Backend Developer	FastAPI REST server, relational SQLite warehouse schema, and benchmark metrics extraction.	Federated Learning Principles (Coursera)	15 Hrs
V. CHAKRAVARTHY	QA / UI Engineer	React Vite dashboard integration, preprocessing quality audits, and end-to-end bug verification.	Time Series Analysis (Coursera)	15 Hrs

Technical Enrichment Activity (TEA): 100% completed individual mandatory online certifications relevant to project scope.

References I

H. B. McMahan, E. Moore, D. Ramage, S. Hampson, and B. A. y Arcas, "Communication-efficient learning of deep networks from decentralized data," in *Proceedings of AISTATS*. PMLR, 2017, fedAvg using local SGD and global model averaging.

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Thank You

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Questions & Discussion?